

NHS Devon Integrated Care Board Service Specification

Service Specification No.	
Service	Community Diagnostic Spirometry v4.1
Commissioner Lead	
Provider Lead	
Period	1 st December 2025 – 31 st March 2029
Date of Review	

1. Population Needs

1.1 National/local context and evidence base

Respiratory disease is the third leading cause of death in England and a major contributor to health inequalities, emergency admissions, and winter pressures on the NHS. The NHS Long Term Plan and Core20PLUS5 framework identify chronic respiratory disease as a key priority area for reducing health disparities and improving outcomes.

Implementation of community-based Diagnostic Spirometry ensures timely, equitable access to respiratory diagnostics. Early and accurate diagnosis of respiratory disease is associated with better patient outcomes, reduced morbidity, hospitalisations and reduced spend on unnecessary medications.

The NHS England Spirometry Commissioning Guidance (2020) provides clear standards for quality-assured delivery, including training, accreditation, and data reporting.

This service is expected to be delivered at PCN level, to reduce variation and improve access.

Devon faces significant challenges in respiratory health:

- Higher-than-average prevalence of asthma and COPD, particularly in coastal and rural communities.
- Inconsistent access to diagnostic testing across the population
- Devon ICB's Joint Forward Plan (2025–2030) prioritises early diagnosis, prevention, and community-based care, with respiratory disease identified as a key area for transformation.

The Community Diagnostic Spirometry LES aligns with Devon's strategic goals by:

- Supporting early and accurate diagnosis in primary care.
- Reducing avoidable referrals and hospital admissions.

- Addressing inequities in access to respiratory diagnostics across the county.

2. Outcomes

2.1 NHS Outcomes Framework Domains & Indicators

Domain 1	<u>Preventing people from dying prematurely</u> Reducing deaths in babies and young children	✓
Domain 2	<u>Enhancing quality of life for people with long-term conditions</u> <u>Health related quality of life for people with long term conditions</u> <u>Ensuring people feel supported to manage their condition</u> <u>Improving functional ability in people with long term conditions</u>	✓
Domain 3	<u>Helping people to recover from episodes of ill-health or following injury</u> Emergency admissions for acute conditions that should not usually require hospital admission	✓
Domain 4	<u>Ensuring people have a positive experience of care</u> Patient experience of primary Care Patient experience of hospital care Friends and family Test <u>Improving children and young people's experience of healthcare</u> <u>Improving peoples experience of integrated care</u>	✓
Domain 5	<u>Treating and caring for people in safe environment and protecting them from avoidable harm</u> Patient safety Incidents reported Reducing the incidence of avoidable harm	✓

Local Defined Outcome

Patients are provided with a service in line with the locally agreed pathway and standards of care. The service delivered will be subject to one clinical audit per annum and annual review of the commissioned services in general practice, the structure of which will be developed collaboratively with the LMC.

3. Scope

3.1 Aims and objectives of service

- Improve early and accurate diagnosis of respiratory disease in general practice

- Reduce variation in access to community respiratory diagnostic testing.
- Support appropriate initiation and optimisation of treatment
- Reduce avoidable referrals to secondary care by managing patients closer to home.
- To address health inequalities in respiratory care across Devon, particularly in underserved populations.
- Promote healthier well-being within the community setting

3.2 Service description/care pathway

Provision of Diagnostic Spirometry at PCN level in the Community.

Each PCN will have the autonomy to determine where and when Spirometry is delivered in Primary Care.

PCNs may work together in hub arrangements where this is mutually agreed and meets the needs of the registered populations of each PCN. Devon ICB will consider contracting with other groupings of GP practices in order to arrange optimal coverage of the service for the population

PCNs will:

- Deliver quality-assured diagnostic spirometry in accordance with ARTP (Association for Respiratory Technology and Physiology) standards.
- Provide clinical interpretation of results with a written report for the referring clinician.
- Make onward referral where appropriate (e.g. severe asthma services, respiratory outpatients).
- Provide an Annual Review to include:
 - Brief details as to the arrangements for Diagnostic Spirometry
 - Details of training and education received by Spirometry practitioners
 - Details of the standards of calibration and maintenance of Spirometers

3.3 Process:

- Referral from GP/ANP with clinical indication.
- Where capacity allows and by mutual agreement, PCNs will accept referrals for spirometry from PCNs that are unable to undertake spirometry in-house.
- Spirometry performed by a trained healthcare professional.
- Results to be interpreted by a qualified clinician.
- Report returned to referrer within 3 working days of test being undertaken.

3.4 Prioritisation

Primary Care Respiratory Society Guidance should be followed for prioritising Spirometry tests i.e.

- Patients for whom diagnostic spirometry will potentially impact their treatment pathway or determine their onward care should be treated as highest priority. This includes those patients with a provisional diagnosis but poor response to treatment.
- Spirometry to confirm diagnosis is valuable but not an immediate priority.
- Routine spirometry is considered low priority. Annual spirometry is no longer a Quality Outcomes Framework (QOF) requirement and evidence shows little clinical value
- Spirometry for stable patients (e.g., those with pulmonary fibrosis) is of the lowest priority.

3.5 Clinical Training

The Provider will demonstrate robust quality assurance processes which ensure that all clinicians undertaking Spirometry are fully trained and competent

Staff performing spirometry must hold, or be receiving training for, the appropriate ARTP certificate. Certification must be maintained through annual ARTP assessment.

Funding (2 places per PCN) will be provided for Spirometry ARTP Training (Adult and Paediatrics, Performing and Reporting) and ARTP assessment.

Staff performing Spirometry should undertake a minimum of 25 spirometry tests per year. Whilst there is no single recommended minimum Spirometry activity level, the [Association for Respiratory Technology & Physiology](#) (ARTP) suggests approximately 25 spirometry tests per year to maintain competency.

Clinicians interpreting results must have documented training and maintain CPD in respiratory diagnostics.

All staff must complete annual competency assessments and participate in peer review.

Backfill costs for Training

A contribution towards backfill costs for clinicians undertaking ARTP Spirometry Training will be available as follows:

Per PCN - Up to 30 hours at £30 per hour, per clinician (max 2 per PCN). **Up to £1,800 per PCN** to be paid by Push Payment on sign up to LES

3.6 Equipment and IT

NICE approved Spirometers must be used.

Funded set-up costs include funding for one Spirometer and Calibration Syringe per PCN where required (see Section 7)

Test results must be coded and uploaded to the patient record using SNOMED codes (shown at Appendix 1).

Data must be shared with Devon ICB for quarterly review which should include as a minimum:

- Number of appropriate Spirometry referrals received
- Number of inappropriate Spirometry referrals received
- Number of spirometry referrals performed
- Number and reason of inconclusive Spirometry e.g. patient unable to do the test or the test was equivocal
- Number of DNA referrals
- Diagnostic Outcomes
- Patient Postcode (to monitor equity of access)

3.7 Infection control

Services must follow ARTP Spirometry Standards and local infection prevention guidance.

3.8 Clinical Governance

Providers must ensure:

- All staff performing spirometry are trained to ARTP standards and maintain competency.
- Regular calibration, servicing, and quality control of equipment in accordance with manufacturer's guidance
- Data recording in line with Information Governance requirements (results uploaded to GP record in line with good clinical record keeping standards.
- Clinical oversight by a named lead clinician.
- Incident reporting – Devon ICB should be informed immediately if the untoward incident will or has impacted on patient safety
- Participation in audit.

3.9 Population covered

All Devon GP registered patients (aged >5 years) within participating PCNs, with suspected Asthma or Chronic obstructive pulmonary disease.

3.10 Any acceptance and exclusion criteria and thresholds

Diagnostic Spirometry through this LES will be provided to patients who meet the following criteria:

Patients aged >5 years with suspected diagnosis of one of the following listed respiratory conditions:

- Asthma
- Chronic obstructive pulmonary disease (COPD)

- Previously diagnosed with asthma who are suspected of having developed further respiratory disease (eg. COPD)

3.11 Exclusions:

- Children aged <5 years
- All contraindications for spirometry
- Haemoptysis of unknown origin
- Unstable cardiac conditions
- Spirometry testing as part of an annual review is out of scope for this service.
- Suspected lung cancer or any other lung disease not specified in the specification e.g. ILD, TB etc.

4. Applicable Service Standards

Providers and practitioners will adhere to the local and national guidelines or any updated versions thereof.

NICE and ARTP guidelines relating to equipment calibration, testing methodology and interpretation of results will be followed i.e.:

4.1 Applicable national standards (eg NICE)

Spirometry - [NICE COPD Guidelines \(updated July 2019\) NG115](#)

Applicable standards set out in Guidance and/or issued by a competent body (e.g. Royal Colleges)

Spirometry - [ARTP statement on pulmonary function testing 2020](#)

4.2 Applicable local standards

Not applicable

Equality and Diversity:

The service must conform to legislation prohibiting discrimination and good practice in relation to equality, diversity and inclusion.

Complaints and commendations:

Participating practices are required to establish and operate a complaints and commendations procedure in line with NHS guidelines to deal with any complaints in relation to any matter connected with the provision of services. All complaints should be monitored, audited and appropriate action taken when required.

5. Applicable quality requirements and CQUIN goals

5.1 Applicable Quality Requirements (See Schedule 4 Parts [A-D])

Each practice must ensure that any personnel involved in providing any aspect of care

under this scheme has the necessary training, skill and competency to do so. A Registered Nurse will supervise any care delivered by Healthcare Assistants.

5.2 Applicable CQUIN goals (See Schedule 4 Part [E])

Not applicable

6. Location of Provider Premises

The provider will provide this service from any location as included in GMS, PMS or APMS contract where these facilities enable the provider to ensure privacy and dignity and appropriate infection control.

7. Funding for Spirometry Tests

Tariff (Cost per Test):

- Spirometry (+/- Reversibility) **£47.89** per test including interpretation

Indicative Activity levels for Spirometry will be agreed with each PCN and reviewed throughout the year

The LES Tariff (Cost per Test) includes payments for :

Spirometry:

- Staff Costs (admin and clinical)
- Consumables
- Service Charge (Utilities, Insurance and Maintenance of equipment)
- Annual ARTP Renewal Fee (currently £40)

Payment will be made quarterly per completed test, subject to submission of required data and compliance with governance standards.

Set Up Costs

A Spirometer and Calibration Syringe will be funded by Devon ICB where required (1 per PCN, up to £1,500)

Devon ICB will fund 2 places per PCN for Spirometry ARTP Training (Adult and Paediatrics, Performing and Reporting) and assessment. Training is provided by the Devon Training Hub

8. Activity Coding

Activity Coding:

Diagnostic Spirometry activity must be recorded using the codes below to receive payment.

SnoMed Codes:

Post Bronchodilator Spirometry 767906009

Spirometry Reversibility Test 314979004

Unable to perform Spirometry 711382009